

GASTROENTEROLOGY CONSULTANTS, P.C.

BRIAN D. CLARKE, M.D.

(317) 621-2100

You are scheduled for a colonoscopy on _____, _____ 200__.

Report to outpatient registration at _____ a.m./p.m. at

Community Hospital North at 7150 Clearvista Pkwy, Phone (317) 621-5193

Carmel Endoscopy Center at 13421 Old Meridian St, Phone (317) 706-1600

You need to be accompanied by a driver for this exam. (A taxi cab or bus is **not** acceptable.)

YOU WILL NEED TO PURCHASE:

MiraLAX® 8.3 oz (238 g) container (No prescription required)

Mix with Gatorade day before prep and refrigerate overnight.

Dulcolax® (bisacodyl) (4 tablets) (No prescription required) – You may delete if you have intolerance

Gatorade® (64 oz bottle) (No red or purple) – Additional Gatorade may be drank for rehydration

Magnesium citrate (1 bottle) (No prescription required) – May not be necessary (see below)

MEDICATION RESTRICTIONS

You may continue to take regular medications on day before procedure except medication that would prevent your colon from being cleaned out. Example: Anti-diarrheal medications, fiber supplements.

Heart or blood pressure medications: Take these on the morning of the colon exam.

Diabetics: Take half of your evening dose of **insulin** on the day before the procedure but **do not take oral diabetes medication or insulin on the day of the colonoscopy** until completed. (If any questions, please contact the doctor that prescribes your insulin.)

Iron supplements (Ferrous sulfate): Stop 1 week before colonoscopy.

Blood thinner: Please discontinue **Coumadin (warfarin), Lovenox (heparin), Aspirin** 3 days before colonoscopy and **Plavix (clopidogrel), Aggrenox (aspirin/dipyridamole), Persantine (dipyridamole), Pletal (cilostazol), Ticlid (ticlopidine)** 5 days before the colonoscopy. Discuss with your prescribing physician for their approval before you stop any medicine that affects blood clotting.

COLON PREP INSTRUCTIONS – The Day Before Your Test

1. **Clear Liquid Diet** (see list below) beginning at breakfast. **No solid food allowed.**
 2. **2:00 p.m. or immediately after work** Take 2 **Dulcolax** tablets by mouth. Patients with diarrhea or working at this time may delete this step. Patients with constipation need to include.
 3. **1-2 hours later** Begin drinking **MiraLAX-Gatorade solution** - 8 oz. every 10-20 minutes until finished or your diarrhea is clear yellow. Drink each glass quickly instead of slowly sipping. Should try to complete in 3-4 hours. Slow down if you develop nausea or vomiting.
 4. **If not clear** Take 2 **Dulcolax** tablets by mouth. This helps to expel any solution still in your colon.
 5. **Midnight** **Please have nothing to eat until colonoscopy is completed.** Clear liquids are encouraged up to two hours prior to your scheduled arrival time. 64 oz. Gatorade is best to replenish electrolytes lost.
- Morning of your colonoscopy at least 3 hours before arrival time (You may need to set an alarm!)**
6. **Early morning** (Example 4 a.m. if arrival time at 7 a.m.) or 8 a.m. if afternoon colonoscopy drink 10 ounces of magnesium citrate. You may skip this step if your diarrhea is clear of solid particles.

CLEAR LIQUID DIET (No red or purple artificial colors)

Jell-O® or gelatin (plain without fruit), coffee or tea, low-salt bouillon/broth, juices without pulp, popsicles, soda-pop, Kool-aid®, Gatorade® & hard candy. No milk products after lunch day before procedure or fruit fiber.

FINAL INSTRUCTIONS

- Bring current medication list with dosages
- If you use a CPAP machine at night for sleep apnea, please bring this with you to the hospital.

Please note: There is a \$75.00 fee for procedures cancelled less than 48 hours (2 business days) before scheduled appointment time

Frequently Asked Questions about Colonoscopy

What is a colonoscopy?

Colonoscopy is a procedure that allows your physician to examine the lining of the rectum and colon for signs of cancer, polyps, or other abnormalities. A flexible tube, about the thickness of an index finger is gently inserted into the anus and advanced through the length of the entire colon. This instrument called a “colonoscope” is equipped with a tiny video camera which sends pictures to a TV screen.

What preparation is required?

The rectum and colon must be completely emptied of stool for the procedure to be effectively performed. When scheduling for the procedure, our office will supply you with information regarding your prep and dietary restrictions. You will also be sedated for the procedure, so you will need to arrange to have someone drive you home afterwards. The sedation could impair your judgment and reflexes for the rest of the day, so you should not drive or operate machinery until the next day.

Why is colonoscopy performed?

Colonoscopy is usually done as either part of a routine screening for cancer, in patients with known polyps or previous polyp removal, to evaluate a change in bowel habits or bleeding, or to evaluate inflammation in the lining of the colon. Colon cancer has become the second leading cause of cancer deaths in the country, killing nearly 60,000 people each year. The good news is that it is also one of the most preventable types of cancer. This is because the majority of colon cancers begin as a small noncancerous growth called a polyp. Polyps grow slowly and can eventually turn into cancer. This transformation can take as long as ten years, during which time you feel perfectly fine, showing no symptoms.

Who should be screened?

Colon cancer affects all races, men and women about equally. Current guidelines suggest screening for all average risk adults over the age of 50. If the exam is normal and there are no other risk factors, repeat examinations should be performed at ten year intervals.

High risk individuals are screened more often. These would include those with symptoms, prior colon cancer or polyps, inflammatory bowel disease, and those with a family history of colon cancer or polyps.

Will my insurance cover a screening colonoscopy?

Since screening colonoscopy is considered routine care, the answer will depend on the insurance plan. Medicare recipients over the age of 50 are now covered for average risk screenings if they have not had a previous colonoscopy within the past ten years. The Medicare Part B deductible is waived in these cases. The State of Indiana passed a law in 2000 that requires most health insurance plans to cover colorectal cancer screening exams and lab tests in accordance with the latest American Cancer Society guidelines. You will need to contact your benefits representative for information specific to your plan.

Will my benefits for routine procedures apply if a polyp is discovered during the course of a screening colonoscopy?

Unfortunately, if a polyp or other abnormality is found during the course of a screening colonoscopy, most insurance companies will not consider the procedure to be a routine screening, but instead cover the service as a diagnostic procedure.